جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : سالمه عامر طه عامر
المحافظة : القاهره **الحي / المدينة :** المرج **العنوان :** مساكن
رقم البطاقة : 27701010113563 **رقم التليفون :** 01028671021 **تاريخ تسجيل الإستمارة :** 14-10-2026
العمر(تاريخ الميلاد) : 01-01-1977 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سالمه عامر طه عامر
National ID: 27701010113563

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
سالمه
عامر طه عامر
٥ ش المدينه المنوره
أول السلام - القاهره
١٩٧٧/٠١
٢٧٧٠١٠١٠١٣٥٦٣
LJ9326912



Name: سالمه عامر طه **MR. :** 29	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 49-year-old premenopausal individual with a left breast invasive ductal carcinoma (IDC) that is estrogen receptor (ER) 3/8, progesterone receptor (PR) 3/8, and HER2 positive (3+). The clinical stage at presentation was cT4bN3b. Neoadjuvant chemotherapy was administered consisting of four cycles of AC followed by four cycles of THP. The patient subsequently underwent a modified radical mastectomy in April 2024, with final pathology of pTisN0/7. Post-operative radiotherapy was completed, and one year of hormonal therapy was finished. Since June 2024 the patient has been on anastrozole (Femara) plus goserelin (Zoladex), with Femara continued for three months and Zoladex ongoing. The patient has completed four cycles of AC and four cycles of THP.

The patient received zoledronic acid (Zometa) today; laboratory results obtained on 5 October 2026 were reviewed and accepted, and calcium supplementation was prescribed. A bone scan is requested.

The patient continues on anastrozole with supportive care and is scheduled to receive goserelin (Zoladex) for three months starting 4 July 2026; the next clinic visit is planned for 8 September 2026.

The patient remains on anastrozole and goserelin, with anastrozole to be continued for an additional three months. The next dose of goserelin is due on 26 September 2026.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

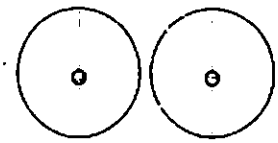
Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : سلمه عامر طه عامر
Sex: Female
3- Medical number: 83:65293
Old Med : 3659-2023

2-ID number: 27701010113563
Patient Age : 47 Y



Family history	Family History -ve	Phone	Phone 01091006663	BMI	BMI	REASON OF MDT	REASON OF MDT POST OPERATIVE
marital status	Marital Status MARRID	NRS Score	NRS Score	GROUP	GROUP C	Date of MDT	Date of MDT 26/05/2024
Co morbidity	Co Morbidity hysterectomy	Menopausal status	Menopausal Status POSTMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
LT locally advanced breast cyst + LN

Mets Workup :
DY node NRI live free

BX	ER	PR	Her 2 Neu	SISH	K.67
IDC	3/8	3/8	3		
BX COMMENT:	ER COMMENT:	PR COMMENT	Her 2 Neu COMMENT:	SISH COMMENT	

T	N	M
4b	3b	0

Sonomammography

PATIENT HISTORY

pt received neoadj 4 AC + 4 taxotere+ HP
LT MRM :free margins
LN 0/8
complete pathological response
YPTis YPN0

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

* Adj. Phesgo (12 cycles)
* E2 level ; IF menopausal , start AI .. If not , start zoladex+ AI
*PORT

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ هنان عزت د/ يحيى اسماعيل	RADIOLOGY	RADIOLOGY د/ نبيل حمزاوي
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name	:الأستاذه / سالمه عامر طه	Request No.	: 13002478
Patient No.	: 27701010113563	Request Date	: 21/04/2024
Path No.	: 1305/24	Verification Date	: 12/05/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data : Left breast cancer, Post NACT .
Nature of the specimen : Left MRM specimens collected at 21/4/2024.

Gross Examination:
-Left MRM specimen measured 24x19x10 cm , with covering skin elipse measurec 22x17 cm.
•Skin involvement: Mild retraction of nipple and areola grossly.
• Tumor extent : An ill defined rubbery greyish white tumor bed site.
• Size: 9.7x7x6cm .
• Distance from margins:0.4 cm away from deep margin .
•Lymph node involvement:8 nodes up to 0.7x0.5 cm.
•Gross designation:
From 1) to 9) Tumor bed site.
10)Nipple and areola , 11)Deep margin , 12)Intramammry node
13) , 14):Axillary nodes.

Microscopic examination:
• Ductal carcinoma insitu , High grade. No invasive component detected.
• Total tumor regression in a previously diagnosed carcinoma.
• Therapy effect in form of fibrosis and histiocytic reaction.
• Skin involvement:Free nipple and areola.
• Resection Margins: Free deep margin.
• Lymph node involvement: Free (0/8) some showed fibrosis (therapy effect).
• Histologic response to therapy:Pathologic complete response.

Diagnosis:
Left breast cancer post NACT , MRM specimen :
-Ductal carcinoma insitu , High grade.

Verified by :
Dr.Marwa mahmoud

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / سالمه عامر طه	Request No.	: 13002478
Patient No.	: 27701010113563	Request Date	: 21/04/2024
Path No.	: 1305/24	Verification Date	: 12/05/2024
Sex	: Female	Financial Categcry	: اورام دار السلام - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

-Total tumor regression in a previously diagnosed invasive carcinoma.
-Therapy effect in the form of fibrosis and histiocytic reaction.
-Free nipple and areola.
-Free deep margin.
-Free lymph nodes: (0/8) , some showed fibrosis (therapy effect).
-Pathologic complete response.
-Stage : ypTis , ypN0.

Prof.Gamal Abdelatty
Dr.Walaa Ghoneim
Dr. Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
Dr.Marwa mahmoud

رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

PATIENT NAME: **SALEMA AMER TAHA AMER** GENDER: **Female** AGE: **46 y**	PATIENT ID #: **10123231900** ORDER #: **2503516,2503517** DATE OF SERVICE: **23-Sep-2023 10:03 AM**

Report: - all studies

CONTRAST-ENHANCED MSCT OF THE CHEST, ABDOMEN AND PELVIS

A reported case of left breast cancer;

1- CHEST findings:

- Large left breast irregular soft tissue mass with breaking down seen within measuring about 7x6.5 cm its maximum axial dimensions. Scattered parenchymal enhancing nodules are seen as well.
- A similar left axillary soft tissue mass is seen abutting the pectoral muscles - measuring 5x4.3 cm ?nodal.
- Associated left breast parenchymal edema and skin thickening.
- Enlarged left axillary level I and II (including inter-pectoral) lymph nodes are seen - the largest measures 18x15.5 mm.
- Enlarged left internal mammary lymph nodes - larger measures 8x10 mm.
- Two right upper lobar millimetric pulmonary nodules are noted.
- No pulmonary masses, consolidation, collapse or cavitary lesions.
- No pathologically enlarged mediastinal or hilar lymph nodes.
- Normal CT appearance of the heart and great vessels.
- No pleural or pericardial effusion.

2- ABDOMEN AND PELVIS findings:

- Enlarged liver (span = 19.5 cm) showing smooth outlines and homogeneous parenchymal attenuation. Segment IVb hypodense focal lesion measuring 12.5 mm in its maximum axial diameter is seen. It is less conspicuous in the delayed phase of the study.
- No dilated biliary radicles.
- Patent homogenously opacified portal vein and its main branches.
- Normal CT appearance of the pancreas, spleen, kidneys, adrenals, aorta and IVC.
- The urinary bladder shows no gross masses, dense stones or diverticula. Mild smooth mural thickening is noted suggestive of cystitis.

CT Scan
[[OCCUPIED_SCAN]]

- Solitary small lymph node with mild cortical thickening is noted 3.2 mm ? Indeterminate.

CONCLUSION:

- Left breast LABC with malignant axillary mass? Malignant lymph node/ satellite and malignant level I and II lymphadenopathy for histopathological confirmation (BIRADS 5)
- Right breast fibrocystic disease with multiple scattered microcalcifications (BIRADS 3).
- Right axillary indeterminate solitary lymph node for FNAC.

Recommendation - all studies

Referral to clinic

Reported By,

Dr. Sara Sakr, MSc **11-Sep-2023 07:14 PM**

Patient Name	: الأستاذه / سالمه عامر طه	Request No.	: 13009850
Patient No.	: 27701010113563	Request Date	: 25/09/2023
Path No.	: SR-1122/23	Verification Date	: 09/10/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer and right axillary lymphadenopathy.
Nature of Specimen: Slide Revision.

Gross Examination:
- Two paraffin blocks labelled by (1456/23) and 5 slides were received.

Microscopic Examination:
Left breast mass :
* Histological type: Invasive duct carcinoma .
* Grade (differentiation+ nuclear pleomorphism+ mitosis):(3+2+1) Grade II.
* DCIS: No.
* Angioinvasion: No.
* Perineural invasion: No.
* IPT:
- ER: Weak nuclear reaction in 3% of tumor cells
(Allred score 3/8).
- PR: Weak nuclear reaction in 3% of tumor cells
(Allred score 3/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Right axillary lymphadenopathy:
Fine niddle aspiration smears and cell block are benign , free of malignant cells .

Diagnosis:
Left breast cancer and right axillary lymphadenopathy, Slide revision:
Left breast mass :
Invasive duct carcinoma , grade II .
IPT:

Verified by :
Rana fathy

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadeema- Cairo- Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / سالمه عامر طه	Request No.	: 13009850
Patient No.	: 27701010113563	Request Date	: 25/09/2023
Path No.	: SR-1122/23	Verification Date	: 09/10/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

- ER: Positive (score 3/8).
- PR: Positive (score 3/8).
- Her2: Positive (score 3+).

Right axillary lymphadenopathy:
Smears and cell block are benign free of malignant cells.

Signature
Dr. Rana Fathy
Dr. Menan Hossam

Verified by :
Rana fathy

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Salma Amer Taha Amer	Date : 08/04/2025
Exam : MAMMOGRAPHY BOTH	Age : 47 Year(s)
Medical ID: 3659/2023	Pat. ID: 42282

DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Patient is 47years old and is seen for follow-up after left MRM

Report:

- Left MRM:
  - Operative bed shows no mass or collection
  - No regional lymphadenopathy.
- Right Breast:
  - Heterogeneous fibroglandular parenchymal densities (ACR c)
  - Diffuse monomorphic punctate foci of calcifications. No suspicious micro calcifications
  - No architectural distortion detected.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Post left MRM status showing no evidence of recurrence (BIRADS 2)
- Stationary benign findings of right breast (BIRADS 2)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
08/04/2025 3:39 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Salma Amer Taha Amer	Date : 30/03/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 46 Year(s)
Medical ID: 3659/2023	

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- ACR:C.
- Bilateral increased parenchymal echogenicity with mottling appearance with scattered cysts.

Left breast:

- Left breast shows UOQ extensive regional microcalcification extending to upper inner quadrant with overlying mild edema pattern and skin retraction.
- Complementary ultrasound revealed left breast infiltrative irregular spiculated isoechoic mass lesion ,invading the overlying skin occupying UOQ extending to UIQ surrounded by area of altered parenchyma ,currently measuring 5X3.3 cm.
- The previously mentioned left axillary mass lesion is currently noted as area of altered parenchyma (melting) measuring 3X1 cm.
- Free left infra and supra-clavicular regions.
- Nonspecific axillary lymph nodes.

Right breast:

- Diffuse scattered right breast punctate calcifications.
- No spiculated lesions or parenchymal distortion.
- No right breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific axillary LN.

Opinion:

- Partial radiological response of the left breast UOQ malignant mass (BIRADS 6)
- Benign findings right breast (BIRADS 2)

Yours truly

Dr.Yomna Tarek,M.Sc.

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــحة والسكـان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Salma Amer Taha Amer	Date : 08/04/2025
Exam : MAMMOGRAPHY BOTH	Age : 47 Year(s)
Medical ID: 3659/2023	Pat. ID: 42282

DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

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Patient is 47years old and is seen for follow-up after left MRM

Report:

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- Right Breast:
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  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Post left MRM status showing no evidence of recurrence (BIRADS 2)
- Stationary benign findings of right breast (BIRADS 2)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
08/04/2025 3:39 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient's Name:	**Salma Amer Taha Amer**
Date:	**2026-05-25**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

RIGHT DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION SONOGRAPHY REVEALED:

CLINICAL SUMMARY:
Left MRM

FINDINGS:

RIGHT BREAST:

Dense fibro-glandular densities, (ACR c).
Diffuse monomorphic round calcifications are still seen, otherwise no spiculations, masses, asymmetry, distortion or suspicious microcalcifications.
Complimentary **U/S** demonstrated no solid or cystic lesion.
Normal skin/nipple/areola complex.
Non-specific axillary nodes.

LEFT MASTECTOMY BED.

Clear mastectomy bed with no masses or fluid collections.
No pathological regional lymph nodes.

CONCLUSION:

- Right breast (BIRADS 2): benign looking calcifications
- Clear left mastectomy bed

Best regards,
Dr. Mahi Mokhtar

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY
INTERNATIONAL

NAME: salma amer
DATE;10/5/2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (15cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with no stones or mud
- *. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

- *Average size, homogenous.*

- **Kidneys: -**
  - *Both are average in size, shape and echogenicity*
  - *No stones or dilated pelvicalyceal system*
  - *No gross lesion*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Average capacity with no stones or diverticular out pouching*

Opinion:

Hysterectomy

Best regards
Dr. NERMEEN RAMADAN FEBR, MD

Ultrasound
[[DOCTYPE:ULTRASOUND]]


جمهورية مصر العربية
بطاقة تحقيق الشخصية
سالمه
عامر طه عامر
٥ ش المدينه المنوره
أول السلام - القاهره
١٩٧٧/٠١
٢٧٧٠١٠١٠١٣٥٦٣
LJ9326912



National ID – Back
[[DOCTYPE:ID_BACK]]